This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Propranolol for the treatment of Anxiety (Situational and Somatic Symptoms)

Patient Group Direction, version 008, issued 30 September 2026. Supersedes Version 007, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Anxiety (Situational and Somatic Symptoms)

Overview

- Anxiety disorders are common, affecting physical and psychological wellbeing
- Physical symptoms include palpitations, tremor, sweating, and gastrointestinal disturbance

Assessment

- Use GAD-7 score to assess severity and impact
- Differentiate generalised anxiety from panic disorder, social anxiety, and PTSD
- Exclude thyroid disease and cardiac causes for physical symptoms

Management

- Cognitive Behavioural Therapy (CBT) is first-line psychological treatment
- Propranolol for symptomatic relief of physical symptoms in situational/performance anxiety
- Propranolol is not first-line for generalised anxiety disorder

Red flags requiring specialist referral

- Suicidal ideation or severe depression
- Severe panic disorder or agoraphobia
- PTSD or trauma-related anxiety
- Comorbid substance misuse

Patient Group Direction

for the supply/administration of

Propranolol 10mg tablets

for the treatment of

Anxiety (Situational and Somatic Symptoms)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer

	• All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Symptomatic relief of situational/performance anxiety (physical symptoms) in adults
Inclusion criteria	Adults aged 18 years and over Physical symptoms of situational anxiety (palpitations, tremor, sweating) Informed consent
Exclusion criteria	Asthma or history of bronchospasm Uncontrolled heart failure Cardiogenic shock Severe bradycardia (heart rate <50 bpm at rest) Hypotension (systolic BP <90 mmHg) Sick sinus syndrome Second-degree or third-degree heart block Phaeochromocytoma (unless already on alpha-blocker) Metabolic acidosis Prinzmetal's angina Concurrent verapamil or diltiazem, oral or intravenous. With a beta-blocker the combination risks severe bradycardia, heart block and hypotension. Known hypersensitivity to propranolol Suicidal ideation, self-harm, or severe depression. Propranolol is cardiotoxic in overdose. Refer. PTSD, severe panic disorder or agoraphobia. Refer. This PGD is for the physical symptoms of situational anxiety only. Comorbid substance or alcohol misuse. Refer. Already taking another beta-blocker. Severe peripheral arterial disease (rest pain, ulceration or critical ischaemia).

	Prolonged fasting, or any other risk of hypoglycaemia, including insulin-treated diabetes with hypoglycaemia unawareness. Pregnancy, or planning pregnancy. Refer. Breastfeeding. Refer to GP.
Cautions	Diabetes (masks hypoglycaemia symptoms) First-degree heart block Portal hypertension Mild peripheral vascular disease (severe peripheral arterial disease excludes) Psoriasis Raynaud's Myasthenia gravis History of anaphylaxis Depression that is not severe and without any suicidal ideation. Supply may proceed with counselling. Severe depression or any suicidal ideation is an exclusion, above. Hepatic/renal impairment Thyroid disease or a possible cardiac cause for the physical symptoms (palpitations, tremor, sweating): where either is suspected, refer rather than supply Elderly Do not stop abruptly
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Propranolol 10mg tablets
Legal category	POM
Storage	Below 25°C, protect from light
Route/method of administration	Oral
Dose and frequency	10 to 40mg as a single dose, taken 30 to 60 minutes before the anxiety-provoking situation. As-required use only: regular daily dosing is not authorised under this PGD.

Quantity to be administered and/or supplied	Up to 28 tablets of 10mg, 280mg of propranolol in total, and no more. Propranolol is cardiotoxic in overdose and the whole supply taken at once must remain below 320mg. One supply per situational event. Review before any repeat. The 40mg strength is not authorised under this PGD.
Maximum or minimum treatment period	As-required use only, before an anxiety-provoking situation. Review before any repeat supply, and in any case at 4 weeks.
Adverse effects	Common: fatigue, cold extremities, bradycardia, GI disturbance, sleep disturbance, dizziness Uncommon: bronchospasm, hypotension, heart failure exacerbation Rare: alopecia, visual disturbance, psoriasiform rash

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • batch number and expiry date of the pack supplied • date of supply, dose, form, route and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Do not stop suddenly, report breathlessness/wheeze, may cause cold hands/feet, not a cure for anxiety - consider psychological therapy, avoid alcohol (additive CNS depression)

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Anxiety disorders
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

GET REAL HEALTH

Version and change record

Version: v008

Issued: 30 September 2026

Supersedes: Version 007, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v007

Issued: 14 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 55: propranolol is authorised for as-required use only, 10 to 40mg as a single dose 30 to 60 minutes before an anxiety-provoking situation. The regular two-to-three-times-daily regimen and the 120mg daily maximum are removed from the dose row; the quantity row states one supply per situational event; the treatment-period row keeps the review before any repeat supply and at 4 weeks and drops the sentences on ongoing use and gradual dose reduction, which described the regular regimen.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document

above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover title corrected from administration to supply; the tablets are supplied.

Treatment-period row: review is now stated as before any repeat supply and in any case at 4 weeks, so it agrees with the 28-tablet single supply in the quantity row.

Cautions: thyroid disease and cardiac causes for the physical symptoms added as a refer-rather-than-supply caution, as the guidance summary already directs.

Records row: batch number and expiry date added, in line with the estate's other POM PGDs.

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Records row: the under-18 record-retention bullet was removed because this PGD does not treat anyone under 18.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document, and any correction notice attached to it.

Change: Version 001 authorised a 40mg arm and printed the same quantity line for both strengths, up to 56 tablets, which at 40mg is 2,240mg of propranolol. Propranolol is cardiotoxic in overdose. The 40mg arm is withdrawn, and the supply is capped at 28 tablets of 10mg, 280mg in total, so that the entire supply taken at once remains below 320mg. The document's own guidance named suicidal ideation, severe depression, PTSD and substance misuse as red flags and none of them was an exclusion. They are now, with another beta-blocker, and the verapamil and diltiazem exclusion covers the oral forms as well as intravenous. This incorporates and withdraws the correction notice of 7 September 2026.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

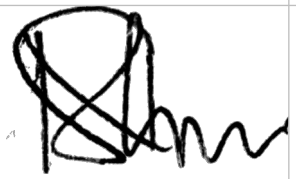
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The expiry cell read 31 October 2026.

Exclusion criteria: the SmPC contraindicates propranolol in severe peripheral arterial disease, after prolonged fasting and in patients prone to hypoglycaemia; the previous version had these as cautions. Now exclusions. Pregnancy and breastfeeding, which the document did not mention, are exclusions with referral.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.